



Oppuna

**Therapy lasts one hour.
Recovery happens during the other 167.**

AI-powered emotional wellness
for the hours around professional care.

Kaushik Itagi · Founder · Investor pitch

PROBLEM

Therapy alone cannot cover the week

Most people who seek care get one hour with a professional—and then face the remaining 167 hours alone.

1**Hour in session**

Guided, structured, scarce

167**Hours between sessions**

Where anxiety spikes, habits form, and progress is won or lost

- ~**197M** people in India live with mental disorders—treatment gaps remain large
- Therapist supply cannot scale to continuous support; waitlists and cost limit frequency
- Between sessions, people need somewhere private to process, track, and cope—today that gap is filled by silence, social media, or cloud apps that harvest intimate data

Takeaway · The gap is the rest of the week—not a shortage of meditation apps.

Sources: The Lancet Psychiatry · NIMHANS NMHS

WHY NOW

Four forces make this investable now

AI maturity

Small on-device models finally run on phones—private AI no longer requires shipping journals to the cloud.

Demand surge

Post-pandemic awareness normalised seeking help; digital mental-health usage is mainstream, not niche.

Therapist shortage

Clinical capacity is constrained by waitlists and cost. Software that helps people between visits can extend scarce professional time.

Paid category exists

Consumers already subscribe to Calm, Headspace, Wysa and peers—digital mental wellness is a paid market.

Takeaway · Timing is right: capable on-device AI + proven willingness to pay in adjacent products.

SOLUTION

What Oppuna actually does

An AI-powered emotional wellness platform—not a chatbot, and not a replacement for clinicians.

Before care

- Name what's happening
- Journal privately
- Arrive prepared for the first session

Day to day

- On-device AI reflection
- Mood tracking and coping tools
- Breathing, grounding, sleep wind-down

Alongside care

- Keep practising what helped
- Export or delete data anytime
- Future: share insights with a therapist—only with consent

Oppuna complements professional care. It does not diagnose, treat, or replace clinicians.

Takeaway · Private tools for reflection, mood, coping, and preparation—kept on the user's device.

USER VALUE

Why someone would choose Oppuna

Hypothesis: users will pay if Oppuna consistently delivers private value they cannot get from a weekly session alone.

Immediate support

Guidance available when distress spikes—not only at the next appointment.

Private reflection

A space to process thoughts without creating a cloud account or sharing journals.

Therapy preparation

Designed to help users stay engaged and arrive better prepared for sessions.

Progress tracking

Mood and journal history that makes patterns easier to notice over time.

Always-on guidance

Coping tools and reflection available on demand, on the user's phone.

Takeaway · Value hypothesis: private, always-available support that makes clinical time more useful—not a clinical outcome claim.

MARKET

A venture-scale category with a clear beachhead

~\$15–35B

TAM — Global digital mental health & wellness apps (illustrative range from public market reports, 2026–2034)

~\$1–3B

SAM — English/Hindi smartphone users seeking paid digital wellness support in India + similar emerging markets

~\$30–80M

SOM (5yr) — Planning range: privacy-conscious adults already in or near professional care (assumption-driven)

- **Beachhead:** Digitally active adults in India using, considering, or returning from professional mental-health care
- **Working assumption:** 10–25M addressable smartphone users × 2–5% paid conversion × ₹2,000–₹3,000 annual ARPU
→ SOM band
- **Expansion path:** Clinician referrals first; employer and institutional programmes later (see roadmap)

Takeaway · Adjacent products already monetise; our beachhead is people already close to professional care.

TAM/SAM/SOM are planning ranges with transparent assumptions—not audited forecasts. Public-health burden ≠ commercial TAM.

COMPETITION

Where Oppuna is different

Adjacent products already earn revenue. Differentiation is architecture and role—not category creation.

Company	Est. reach	Paid model	Core focus	Oppuna difference
Wysa	6M+ users (reported)	~\$75-\$99/yr Premium	AI CBT chat + coaching	On-device option; no account required
Woebot	Pioneer; consumer app wound down 2025	Enterprise / clinical	Structured CBT check-ins	Direct-to-consumer + privacy-first
Headspace	85M+ downloads; ~2M paid	~\$70/yr	Meditation + content	Reflection tools around care, not a content library
Calm	~4M paid (est.)	~\$70/yr	Sleep & meditation	Mood, journal, AI reflection—not sleep audio
Youper	2-3M users (reported historically)	~\$70/yr	AI mood coaching	Local-first storage; clinician-adjacent design

Oppuna difference: on-device AI option · journals stay local by default · no account required · crisis routing without uploading content · built to complement clinicians, not replace them.

Takeaway · Paid demand exists in the category; Oppuna competes on privacy architecture and clinician adjacency.

Figures are public estimates from industry reports and company claims; see appendix for diligence notes.

BUSINESS MODEL

Who pays, what they pay, how we expand

	Who pays	What they pay	What is included
Free	End users	₹0	Journal, mood, breathing, grounding, safety routing, local storage
Premium	End users	₹299/mo or ₹2,499/yr (<i>proposed</i>)	Advanced on-device AI reflection, deeper insights, export tools
Future B2B	Clinics / employers / marketplace	TBD after pilots	Referrals, wellness programmes, verified provider discovery

- **Conversion funnel (hypothesis):** Install → first journal/mood check-in → weekly return → Premium trial → paid
- **Near-term payer:** Individuals already close to professional care
- **Later expansion:** Clinic partnerships → campus pilots → employer programmes (roadmap)

Takeaway · Simple consumer freemium first; B2B only after retention and willingness-to-pay are measured.

Pricing and conversion rates are proposed and unvalidated. Private emotional data is never sold or used for ads.

GO-TO-MARKET

Three priority acquisition channels

Focus capital on channels that can prove retention and referrals before broad paid spend.

1. Psychologist referrals

Partner with clinicians who want patients to stay engaged between appointments. Measure referral → install → activation.

2. Organic content

Educational wellness content and SEO that attracts privacy-conscious seekers. Compounding, measurable CAC over time.

3. University & community pilots

Campus and peer-community programmes for structured cohorts, feedback, and early retention learning.

Referral → install

Activated users

D7 / D30 retention

Content CAC

Pilot completion

Takeaway · Win trust through clinicians, content, and campuses—employer/institutional scale comes later on the roadmap.

Evidence to date

100+

Android downloads in the first three days on Google Play

Play Console · Downloads, not retained users or MAU

11

Psychiatrists interviewed

Qualitative discovery—not clinical endorsement or outcome evidence.

Product milestones

- Live on Google Play
- iOS under App Store review
- Production Android + on-device AI
- Public site + privacy policy
- Open development on GitHub

Takeaway · Early signal only: shipped product, initial downloads, and clinician interviews—retention and revenue still to prove.

THE ASK

Raising \$750K to validate growth—not rebuild the product

\$750K · Pre-seed

~18 months of runway. Capital prioritises acquisition, validation, partnerships, and marketing.

Use of funds	%	Focus
User acquisition & marketing	42%	Content, ASO, paid tests, community
Product validation	20%	Retention experiments, Premium tests, UX
Partnerships	18%	Clinician referrals, campus pilots
Engineering	8%	Reliability, app size, on-device AI polish
Clinical / legal / ops	12%	Safety review, compliance, runway

Why this team

Kaushik Itagi — Founder who shipped Oppuna end-to-end (mobile, on-device AI, privacy architecture). Lived experience with anxiety and CBT informed the product thesis.

With this round: growth lead, clinical advisors, trust-focused design—not a large engineering team.

Must prove: retention, willingness to pay, and referral conversion.

Takeaway · Funding is for users, validation, and partnerships—the product foundation already ships.

ROADMAP

What we will prove next

NOW

Foundation

- Android live on Play
- iOS through review
- Clinician discovery ongoing
- Store & brand assets shipping

12 MONTHS

Monetisation

- ~5K MAU target
- Paid conversion learning
- Campus + community loops
- Consent-based provider discovery pilots

6 MONTHS

Activation

- Structured beta (500+)
- Measure D7/D30 retention
- First psychologist referral pilots
- Premium price tests

24 MONTHS

Scale

- 10–25K MAU ambition
- 1,000+ paid subs target
- Employer & institutional programmes
- Opt-in therapist collaboration

Takeaway · Milestones are validation gates—retention and payment first; institutional scale second.

Operating targets—not guarantees. Contact: support@oppuna.com · oppuna.com · github.com/kaushik-i-b/oppuna

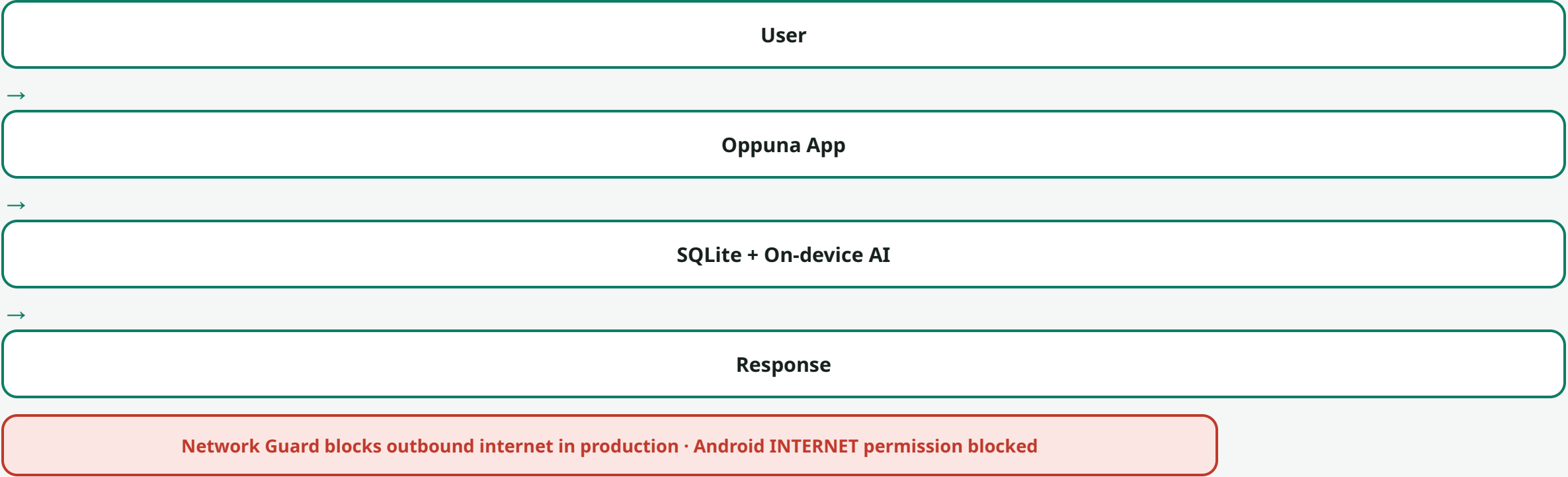
DILIGENCE DETAIL

Appendix

Architecture, safety, unit economics, and sources—kept out of the core story.

PRODUCT ARCHITECTURE

Privacy by architecture



Crisis routing

Local pattern matching pauses coaching and shows regional helplines—without uploading journals.

On-device LLM

Optional Qwen GGUF via llama.rn for local inference.

Guided fallback

SHIPPING TODAY

Product surface (for diligence)

AI companion

Supportive chat with local safety routing

Mood & insights

Check-ins, tags, weekly patterns

Journal

Daily, gratitude, thought records

Breathing & grounding

Guided calm exercises

Sleep wind-down

Checklist + device TTS

Voice notes

Private local audio

APPENDIX

ORIGIN

Founder insight and CBT context

Kaushik experienced anxiety personally; CBT helped—but progress also came from recording thoughts, noticing patterns, and practising healthier responses *between* sessions. That experience shaped Oppuna’s product thesis.



CBT-inspired reflection may inform design. Oppuna does not provide CBT or psychotherapy. Future exercises will be reviewed with qualified professionals.

\$750K

Detailed use of funds

User acquisition & marketing — ~\$315K

Content, social, ASO, paid acquisition tests, community programmes.

Product validation — ~\$150K

Retention experiments, Premium tests, onboarding UX, instrumentation.

Partnerships — ~\$135K

Psychologist referral programmes, university and community pilots.

Engineering — ~\$60K

Reliability, app-size reduction, on-device model polish—smaller allocation.

Clinical / legal / ops — ~\$90K

Safety review, advisors, compliance, entity, insurance, runway buffer.

APPENDIX

OPERATIONS

Crisis routing & app size

Crisis routing

Local regex/pattern matching before generative replies. High-risk categories open a crisis screen with Tele-MANAS (14416 / 1-800-891-4416) and Emergency 112. Not clinical triage.

App size

~1.1 GB today mainly from bundled on-device Qwen GGUF via Play Asset Delivery. Plan: post-install model download, smaller quant, optional packs—target sub-200 MB core experience.

Marketplace & illustrative revenue

Verified Care Marketplace (proposed)

Credentialed psychiatrists, psychologists, institutions. Listing fees, booking fees, sponsored education—never targeted from journals or emotions.

Illustrative Premium math

At ₹2,499/year digital Premium:

1,000 payers $\approx$ ₹25L/yr

10,000 payers $\approx$ ₹2.5Cr/yr

Not a forecast.

APPENDIX

EVIDENCE

Sources & competition diligence

- The Lancet Psychiatry — India mental-disorder burden
- NIMHANS NMHS — treatment gaps
- Tele-MANAS — national tele-mental health
- Competitor reach/pricing: public company claims and industry summaries (Business of Apps, product sites)—verify at diligence
- Oppuna traction: Google Play Console downloads; founder-led psychiatrist interviews (qualitative)

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